



This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practise under it.

Healthcare professionals must work within their own competence. This PGD does not remove professional obligations or accountability.

ISSUED, VALID FROM 20 SEPTEMBER 2026

Patient Group Direction

for the administration of Havrix Monodose, Havrix Junior Monodose, Avaxim or Avaxim Junior (inactivated hepatitis A vaccine) for Protection against Hepatitis A

Protection against Hepatitis A

From age 1 year onwards.

Patient Group Direction, version 001, issued 20 September 2026. Havrix Monodose, Havrix Junior Monodose, Avaxim and Avaxim Junior. Individuals aged 1 year and over, for travel and non-travel risk.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

What this PGD is for

The administration of inactivated hepatitis A vaccine to an individual aged 1 year or over who is at increased risk of hepatitis A through travel, lifestyle, medical condition or occupation, by a registered pharmacist or registered pharmacy technician in a community pharmacy, without a prescription. Supply and administration under this PGD is a private service.

What it authorises

- A first dose of monovalent hepatitis A vaccine: Havrix Monodose or Avaxim at 16 years and over; Havrix Junior Monodose or Avaxim Junior from 1 year to 15 years inclusive.
- A second (booster) dose 6 to 12 months after the first, or later within the window stated for the product, to complete the course for long-term protection.

What it does not cover

- Children under 1 year. None of the products is licensed below 1 year.
- Hepatitis B vaccination, or the combined hepatitis A and B vaccine (Twinrix). Those are under the Hepatitis A and B (Travel) PGD.
- Combined hepatitis A and typhoid vaccine (ViATIM), which is not authorised by any Get Real Health PGD.
- Post-exposure use, that is a contact of a case of hepatitis A or a person exposed in an outbreak. Refer to the GP or the local Health Protection Team the same day.
- Serology to confirm immunity, before or after vaccination.
- A patient who has had a confirmed anaphylactic reaction to a previous dose of hepatitis A vaccine or to any component, including neomycin.



Summary of Green Book chapter 17 guidance for hepatitis A vaccination

UKHSA, *Immunisation Against Infectious Disease (the Green Book)*, chapter 17, Hepatitis A, chapter dated 12 January 2024, page updated 15 January 2024, and the *Summaries of Product Characteristics for Havrix Monodose, Havrix Junior Monodose, Avaxim and Avaxim Junior*, current versions. Summarised 20 September 2026.

The disease

Hepatitis A is a viral infection of the liver. The disease is generally mild but severity increases with age. Asymptomatic infection is common in children; jaundice occurs in 70 to 80 per cent of those infected as adults.

There is NO chronic persistent state and chronic liver damage does not occur. Fulminant hepatitis is rare.

Spread is faecal-oral, person to person or through contaminated food or drink, including shellfish, frozen berries, dates and salad vegetables. Incubation is usually around 28 to 30 days.

Most adolescents and adults in the UK remain susceptible throughout life.

The highest risk areas for UK travellers are the Indian subcontinent, the Middle East, Africa and South East Asia, and the risk now extends to Eastern Europe.

Who the chapter recommends it for

TRAVEL: recommended for those aged one year and over travelling to areas of high or medium endemicity. Give preferably at least two weeks before departure, but it can be given up to the day of departure.

Immunisation is NOT generally considered necessary for Northern or Western Europe including Spain, Portugal and Italy, nor North America, Australia or New Zealand.

People with chronic liver disease of any cause, and consider it in chronic hepatitis B or C.

People with haemophilia receiving plasma-derived clotting factors, who should be immunised SUBCUTANEOUSLY.

Gay, bisexual and other men who have sex with men, and people who inject drugs.

Occupational: laboratory workers who may be exposed, staff and residents of some large residential institutions, sewage workers at risk of repeated exposure to raw sewage, and people working with susceptible primates.

Routine immunisation is NOT indicated for most healthcare workers.

Schedule

Two doses, the second 6 to 12 months after the first.

A single dose protects for up to 12 months. The second dose at 6 to 12 months gives immunity for AT LEAST 25 YEARS.

A booster 25 years after a completed course is generally not needed, except for those at ongoing risk or after exposure to a case.

A delayed second dose does not mean restarting: successful boosting occurs even when the second dose is delayed for several years. The Havrix Monodose SmPC states that it is unnecessary to restart the primary schedule if the booster is given within 5 years of the first dose, and Havrix Junior Monodose within 3 years; Avaxim allows the booster up to 36 months after the first dose and Avaxim Junior between 6 months and 15 years after it.

Where rapid protection is needed, a single dose of MONOVALENT vaccine is recommended. The monovalent products contain more hepatitis A antigen and protect more quickly than the combined hepatitis A and B or hepatitis A and typhoid products.

PHENYLALANINE: Avaxim and Havrix contain phenylalanine in small amounts. Advise a patient, parent or carer with phenylketonuria to account for it in meal planning on the day.

Contraindications, pregnancy and immunosuppression

Do not give to anyone who has had a confirmed anaphylactic reaction to a previous hepatitis A-containing vaccine or to any component. Check the SmPC for excipients; the products may contain trace neomycin.

Minor illness without fever or systemic upset is not a reason to postpone. Acute severe febrile illness: postpone until recovered.

PREGNANCY: hepatitis A vaccine may be given when clinically indicated. The vaccines are inactivated. The Avaxim SmPC advises use in pregnancy only when clearly necessary after an assessment of risks and benefits.

BREASTFEEDING: no contraindication. The Avaxim SmPC states both Avaxim products may be used during breastfeeding.



IMMUNOSUPPRESSION, including HIV: the vaccine may be given, though seroconversion and antibody titres may be lower and relate to CD4 count. Where the patient needs to know that they responded, serology is required.

ANTICOAGULATION: someone on stable anticoagulation, including warfarin where INR testing is up to date and the latest INR was below the top of the range, can be vaccinated intramuscularly using a 23 gauge or finer needle, with firm pressure without rubbing for at least two minutes. Where there is a bleeding disorder, the subcutaneous route may be used.



The Patient Group Direction

How to use this PGD

Which product, and why the age matters

- **FOUR PRODUCTS, TWO STRENGTHS.** The adult products (Havrix Monodose 1440 ELISA units in 1.0 mL; Avaxim 160 units in 0.5 mL) are for 16 years and over. The paediatric products (Havrix Junior Monodose 720 ELISA units in 0.5 mL; Avaxim Junior 80 units in 0.5 mL) are for 1 year to 15 years inclusive. The paediatric products contain half the antigen of the adult ones and the packaging is similar. Check the age against the product in hand before every dose.
- **THE DOSE VOLUMES DIFFER.** Havrix Monodose is 1.0 mL. Every other product here is 0.5 mL. Do not carry a volume across from one product to another.
- Havrix and Avaxim are both UK-licensed inactivated hepatitis A vaccines and either may be used. Use whichever the pharmacy holds; do not delay vaccination to obtain the other.
- A course started with one product should be completed with the same product where possible. Where it is not, the booster may be given with the other product: the Avaxim SmPC supports Avaxim as the booster after a different inactivated hepatitis A vaccine.

The schedule

- **ONE DOSE** gives protection from about 2 weeks and lasts at least a year. It can be given up to the day of departure; some protection develops before antibody is detectable, so a late dose is worth giving.
- **A SECOND DOSE** at 6 to 12 months completes the course and gives protection for at least 25 years. No further boosters are needed for immunocompetent people.
- **A LATE SECOND DOSE STILL COUNTS.** Do not restart the course. Havrix Monodose: the booster may be given up to 5 years after the first dose without restarting. Havrix Junior Monodose: up to 3 years. Avaxim: up to 36 months. Avaxim Junior: between 6 months and 15 years. Beyond those windows a booster is generally still effective; where it falls outside the licence, record that as an informed off-label decision rather than declining.
- Someone who completed a two dose course, at any time in the past, does not need a further dose under this PGD unless they are at ongoing risk and it has been 25 years. Ask, and record what they tell you.

Two things this PGD deliberately does not do

- It does not cover hepatitis B or the combined vaccines. A traveller who also needs hepatitis B is seen under the Hepatitis A and B (Travel) PGD, which authorises Twinrix and Engerix B as well as the products here, so that one consultation covers both.
- It does not cover post-exposure use. Hepatitis A vaccine given to a contact of a case is a public health intervention with its own timing rules and, for some contacts, immunoglobulin. That is for the Health Protection Team and the GP, the same day.

Observation

- Observe every patient for 15 minutes after vaccination, seated, and record that the observation period was completed.



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Patient Group Direction

for the administration, for Protection against Hepatitis A, of:

Hepatitis A vaccine

Patient Group Direction for the administration of Havrix Monodose, Havrix Junior Monodose, Avaxim or Avaxim Junior for protection against hepatitis A in individuals aged 1 year and over.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC.
- All users must be trained to the National Minimum Standards and Core Curriculum for Immunisation Training, and be competent in intramuscular vaccination technique in adults and children.
- All users must be competent in the recognition and immediate management of anaphylaxis, and hold current basic life support training.
- All users must be competent in assessing consent in children and young people, including parental responsibility and Gillick competence. This PGD covers children from 1 year.
- All users must know that the four products come in two strengths by age and that Havrix Monodose is a 1.0 mL dose while the other three are 0.5 mL. Confirm the age and the product in hand before drawing up.
- All users must be able to take a travel and risk history sufficient to establish the indication, and to recognise a post-exposure situation and refer it.
- All users must be competent in vaccine handling and cold chain management, including the action to take on a cold chain excursion.
- All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required. All users must work in compliance with the SOPs of their own employer, and must practise only within the bounds of their own competence.
- All users must be familiar with the current Summary of Product Characteristics for every product named in this PGD, and with current BNF and national guidance for the condition.
- All users must be up to date with MHRA drug safety alerts and recalls relevant to these products.
- All users must be up to date with their CPD requirements and appraisal.
- All users must hold appropriate professional indemnity covering this service.
- All users must understand capacity and consent, including the Mental Capacity Act 2005, and must be able to assess consent in children and young people where this PGD covers them.

The PGD

Indication	Active immunisation against hepatitis A in individuals aged 1 year and over who are at increased risk through travel, lifestyle, a medical condition or occupation, in accordance with Green Book chapter 17 and the current Summary of Product Characteristics for the product used. Supply and administration under this PGD is a private service.
Inclusion criteria	• Aged 1 year and over. Adult products at 16 years and over; paediatric products from 1 year to 15 years inclusive. • Travel to an area of moderate or high hepatitis A endemicity, which in practice is anywhere outside northern and western Europe, North America, Australia and New Zealand. Check the destination on NaTHNaC TravelHealthPro where there is doubt.

	• Or a non-travel risk factor: chronic liver disease including chronic hepatitis B or C; haemophilia or receipt of plasma-derived clotting factors; people who inject drugs; gay, bisexual and other men who have sex with men; or an occupational risk such as laboratory work with the virus, sewage work, or work with susceptible primates. • For a second dose: a first dose of an inactivated hepatitis A vaccine 6 months or more ago, with the product and date known or reliably reported. • Valid informed consent obtained. Where the individual is under 16, from a person with parental responsibility, or from the young person where assessed as Gillick competent, with the basis of that assessment recorded. A parent accompanying a child does not automatically hold parental responsibility; ask. • The patient understands this is a private service and what it costs. • No exclusion criterion present.
Exclusion criteria	Refer, do not vaccinate, where any of the following applies. • Under 1 year of age. • Confirmed anaphylactic reaction to a previous dose of any hepatitis A-containing vaccine, or to any component of the product to be used. Havrix and Avaxim may contain trace NEOMYCIN, so a neomycin hypersensitivity excludes every product under this PGD. • Previous hypersensitivity reaction following a hepatitis A-containing vaccine. • Acute severe febrile illness. Postpone until recovered. Minor illness without fever is not a reason to defer. • A completed two dose course of hepatitis A vaccine, unless the patient is at ongoing risk and 25 years have passed. There is nothing to add. • Post-exposure use: a contact of a case, or exposure in an outbreak. Refer to the GP or the Health Protection Team the same day. • The patient requires proof of immunity, since serology is out of scope. • Under 16 and valid consent cannot be obtained from a person with parental responsibility, and the young person is not assessed as Gillick competent.
Cautions	Pregnancy. Hepatitis A vaccine may be given where clearly indicated. The vaccines are inactivated. Havrix is preferred; the Avaxim SmPC advises use only when clearly necessary after an assessment of risks and benefits, and that assessment must be recorded. Breastfeeding. No contraindication; both Avaxim products may be used during breastfeeding and there is no established concern with Havrix. Record the decision. Immunosuppression, including HIV. The response may be reduced and relates to CD4 count. Vaccination is still recommended. Where the patient needs to know whether they responded, that requires serology and is outside this PGD: counsel and refer rather than assuming protection. Bleeding disorders, thrombocytopenia or anticoagulation. Intramuscular injection can usually still be given using a fine needle, 23 gauge or finer, with firm pressure without rubbing for at least 2 minutes. Deep subcutaneous injection is the fallback; the Green Book advises the subcutaneous route for people with haemophilia receiving plasma-derived clotting factors. Phenylalanine. All four products contain phenylalanine: Avaxim and Avaxim Junior 10 micrograms per dose, Havrix Junior Monodose 83 micrograms per dose, Havrix Monodose as stated in its SmPC. Relevant only in

	phenylketonuria; advise the patient or carer to account for it in meal planning on the day. Latex. The needle shield of the attached-needle presentation of adult Avaxim may contain natural rubber. Check the presentation in hand before use in a latex-sensitive patient. The Avaxim Junior needle shield is polyisoprene. Havrix presentations are described as free of natural latex; check the current leaflet before reassuring a latex-allergic patient. The patient also needs hepatitis B protection. This is not an exclusion, but the combined vaccine under the Hepatitis A and B (Travel) PGD covers both in one course and is usually the better choice. Tell the patient, record the decision, and where hepatitis A alone is given make clear that it gives no protection against hepatitis B or C. A patient presenting shortly before departure. Give the dose: some protection develops before antibody is detectable and it can be given up to the day of travel. Be explicit that full protection comes after about 2 weeks.
Anaphylaxis and observation	Adrenaline (epinephrine) 1 in 1,000 injection must be immediately available whenever a vaccine is administered under this PGD, together with a written anaphylaxis protocol consistent with current Resuscitation Council UK guidance, and a telephone. All staff administering vaccines must be trained in the recognition and immediate management of anaphylaxis and must hold current basic life support training. OBSERVE EVERY PATIENT FOR 15 MINUTES AFTER VACCINATION. Vaccinate seated. Anxiety-related reactions including vasovagal syncope, hyperventilation and transient visual disturbance or paraesthesia can occur before or after any injection. Procedures must be in place to prevent injury from a faint, which is common particularly in adolescents.
Actions if the patient is excluded or declines	• Discuss the reason for exclusion with the patient and make sure they understand it. • Give food and water hygiene advice for the destination regardless of whether vaccine is given. • Refer to the GP, a travel clinic or the Health Protection Team as appropriate, and make the urgency explicit where it is a post-exposure situation. • Where hepatitis B is also needed, offer the Hepatitis A and B (Travel) consultation instead of two separate ones. • Document the reason for exclusion, the advice given and the decision reached. Inform or refer to the GP as appropriate.
Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	• That valid informed consent was given, and from whom. Where the individual is under 16, the name and relationship of the person with parental responsibility, or the basis of the Gillick assessment. • Patient name, address, date of birth, and the GP with whom they are registered. • Destination and departure date, or the non-travel risk factor that made vaccination appropriate. • Which product was given, Havrix Monodose, Havrix Junior Monodose, Avaxim or Avaxim Junior, with batch number and expiry date. • Dose number in the course (first or second), and for a second dose the product and date of the first where known.

	• Date of administration, dose volume, route and anatomical site. • The date the second dose is due, and that the patient was told. • Where the second dose was given outside the licensed window, that this was an informed off-label decision. • Name and registration number of the healthcare professional administering. • That the 15 minute observation period was completed. • Advice given, including food and water hygiene advice and advice given if the patient is excluded or declines. • Details of any adverse drug reactions and the actions taken. • That the vaccine was administered under this PGD. Records signed, dated, legible and contemporaneous. Adults 18 and over: 8 years. Under 18s: until the 25th birthday, or the 26th where the patient was 17 when the course finished. The patient must also be given a written record of the product, the date and when the second dose is due.
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The medicine

Name, form and strength	• HAVRIX MONODOSE (GSK), hepatitis A vaccine (inactivated, adsorbed), 1440 ELISA units in 1.0 mL, suspension for injection in pre-filled syringe. 16 years and over. • HAVRIX JUNIOR MONODOSE (GSK), 720 ELISA units in 0.5 mL, suspension for injection in pre-filled syringe. 1 year to 15 years inclusive. • AVAXIM (Sanofi), hepatitis A vaccine (inactivated, adsorbed), 160 units in 0.5 mL, suspension for injection in pre-filled syringe, PL 23228/0006. 16 years and over. • AVAXIM JUNIOR (Sanofi), 80 units in 0.5 mL, suspension for injection in pre-filled syringe, PL 23228/0007. 1 year to 15 years inclusive. • THE DOSE VOLUMES DIFFER: 1.0 mL for Havrix Monodose, 0.5 mL for the other three. Confirm the age and the product in hand before drawing up. • All four may contain trace neomycin and contain phenylalanine. None contains a live organism.
Legal category	POM.
Dose and frequency	• FIRST DOSE: one dose of the product for the patient's age. Protection from about 2 weeks, lasting at least 12 months. • SECOND DOSE: one dose 6 to 12 months after the first, which gives protection for at least 25 years. No further routine boosters for immunocompetent people. • LATE SECOND DOSE: give it; do not restart. Within licence up to 5 years after the first dose for Havrix Monodose, up to 3 years for Havrix Junior Monodose, up to 36 months for Avaxim, and between 6 months and 15 years for Avaxim Junior. Beyond the licensed window, record an informed off-label decision. • A patient who turns 16 between doses has the second dose with the adult product for their age at the time of that dose.
Quantity to be administered	One dose per patient per attendance: 1.0 mL of Havrix Monodose, or 0.5 mL of Havrix Junior Monodose, Avaxim or Avaxim Junior. This PGD does not permit supply of vaccine to the patient for administration elsewhere.
Route and method of administration	Intramuscular injection: deltoid in adults, adolescents and older children; anterolateral thigh in young children. Never into the gluteal muscle, and never intravascularly or intradermally: the response is unreliable. Shake well before use and inspect; do not administer if the appearance differs from that

	described in the SmPC. For a patient with a bleeding disorder, use a fine needle with firm pressure, or the subcutaneous route. Where another vaccine is given at the same visit, use a separate limb where possible, or sites at least 2.5 cm apart, and record the site of each. Do not mix with any other vaccine in the same syringe.
Co-administration	May be given at the same time as other inactivated or live travel vaccines, including typhoid and yellow fever, at a separate site with a separate syringe. Seroconversion is not affected. Where the patient also needs hepatitis B, see the Hepatitis A and B (Travel) PGD rather than giving Twinrix under this one.
Storage and cold chain	Store at +2C to +8C in the original packaging to protect from light. DO NOT FREEZE. A vaccine that has been frozen, or is suspected of having been frozen, must be quarantined, labelled and not administered: freezing separates the antigen from the aluminium adjuvant, which reduces potency and can increase reactogenicity. COLD CHAIN EXCURSION: vaccine exposed to conditions outside the stated range must be quarantined and risk assessed in accordance with UKHSA Vaccine Incident Guidance before any further use. Neither Avaxim SmPC contains temperature excursion stability data, so there is no permitted out-of-fridge period for those products.
Disposal	Dispose of used syringes, needles and any discharged vaccine in a UN-approved puncture-resistant sharps container in accordance with local arrangements and HTM 07-01.
Adverse effects	Very common: injection site pain and redness, fatigue, headache, and in children irritability. Common: fever, malaise, injection site swelling or induration, gastrointestinal upset, drowsiness and loss of appetite. Uncommon: dizziness, myalgia, rash, influenza-like illness, vomiting. Rare or very rare: paraesthesia, arthralgia, urticaria, pruritus, lymphadenopathy, chills. Reported post-marketing: anaphylaxis, Guillain-Barre syndrome, convulsions, vasculitis, thrombocytopenia, erythema multiforme and angioedema. Consult the current SmPC for the product used.

Information for the patient

Written information	Supply the patient information leaflet for the product given, and a written record of the vaccine, the batch number, the date, and the date the second dose is due. Advise the patient to keep it, because the brand determines how the course is completed elsewhere.
Counselling	• One dose protects you for about a year, starting around two weeks from today. The second dose, in 6 to 12 months, protects you for at least 25 years. Come back for it; we will book it now. • If you miss the second dose date, come anyway. The course does not need restarting. • The vaccine does not cover everything you can catch from food and water. Drink bottled or boiled water, avoid ice, salads, shellfish and food that has been standing, and wash your hands. • Some soreness, tiredness, headache or mild fever in the first day or two is common and settles on its own. • Where hepatitis B was discussed: this vaccine does not protect against hepatitis B or C. The precautions for those are avoiding unprotected sex, unsterile tattooing, piercing and acupuncture, and not sharing needles or razors.
Follow-up	Book the second dose at this appointment, 6 to 12 months from today. Seek medical advice for jaundice, dark urine or pale stools after travel, whatever the vaccination status. For a routine query about the vaccine or the schedule,

	contact the pharmacy.
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Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions.
- UKHSA, Immunisation Against Infectious Disease (the Green Book), chapter 17, Hepatitis A, chapter dated 12 January 2024, page updated 15 January 2024, and the Summaries of Product Characteristics for Havrix Monodose, Havrix Junior Monodose, Avaxim and Avaxim Junior, current versions. Summarised 20 September 2026.
- The current Summary of Product Characteristics for each product named in this PGD, [medicines.org.uk](https://www.medicines.org.uk).
- British National Formulary, current edition.



Signed on behalf of Get Real Health

Authorised by the Medical Director and the Head Pharmacist named below. This version is not valid without both signatures.

Version	v001
Supersedes	undefined
Valid from date	20 September 2026
Expiry date	31 July 2027

Doctor	Name: Nitin Shori Job title: Medical Director, Get Real Health GMC: 6047293 Signature:  Date: 20 September 2026
Pharmacist	Name: Chris Pilkington Job title: Head Pharmacist, Get Real Health GPhC: 2046322 Signature:  Date: 20 September 2026

Both authorising signatories reviewed this version together on 20 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs.

PGD adoption and authorisation by the employer or clinical lead

The employer has ensured that the person using this PGD has the knowledge and skills to safely provide the service, and that appropriate governance is in place. This section must be signed by the superintendent or clinical lead responsible for this service.

To be completed by the adopting pharmacy. These signatories are the adopting organisation own signatories and must not be pre-filled by Get Real Health.

Superintendent / clinical lead	Name: Job title and organisation: Signature: Date:
Professional group signatory	Name: Job title and organisation:



Signature:

Date:



Agreement to practise by the registered healthcare professional

By signing below you confirm that you agree to the contents of this PGD and that you will work within it. A PGD does not remove the inherent professional obligations or accountability of the individual practitioner. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of the pharmacy or clinic premises to which this PGD relates

Premises name	
Address	 Postcode:
ODS code, if applicable	

Registered healthcare professionals authorised to work under this PGD at those premises

The employing organisation must keep this page, or an equivalent local register, and must be able to produce it on request. A practitioner who has not signed against the current version is not authorised to work under it.

Name of healthcare professional	Job title Registration number Signature Date
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Valid from date	20 September 2026
Expiry date	31 July 2027

Change history

Version	v001
Date	20 September 2026
Changes	• New standalone PGD. Hepatitis A vaccination was previously authorised only within the Hepatitis A and B (Travel) PGD and the Travel Health core PGD, and pharmacists looking for a hepatitis A PGD could not find one. This document brings the hepatitis A content of those two documents together under its own title, with its own consultation tool. • Products and ages are unchanged from those documents: Havrix Monodose and Avaxim at 16 years and over, Havrix Junior Monodose and Avaxim Junior from 1 year to 15 years inclusive. Hepatitis B, Twinrix and ViATIM remain outside this PGD. • The late second dose is stated per product from the current SmPCs: Havrix Monodose within 5 years, Havrix Junior Monodose within 3 years, Avaxim within 36 months, Avaxim Junior between 6 months and 15 years, with a recorded off-label decision beyond those windows rather than a refusal. • Post-exposure use is an explicit exclusion with same-day referral to the GP or Health Protection Team. • Records include the dose number, the product and date of the first dose where a second is given, the date the second dose is due, and completion of the 15 minute observation period.

Appendix 1: Key references

- UKHSA. Immunisation Against Infectious Disease (the Green Book), chapter 17, Hepatitis A, chapter dated 12 January 2024.
- Summary of Product Characteristics for Havrix Monodose, Havrix Junior Monodose, Avaxim (PL 23228/0006) and Avaxim Junior (PL 23228/0007), current versions, electronic Medicines Compendium.
- National Travel Health Network and Centre, travelhealthpro.org.uk, hepatitis A factsheet and country information pages.
- UKHSA Vaccine Incident Guidance.
- National Minimum Standards and Core Curriculum for Immunisation Training.
- Resuscitation Council UK. Emergency treatment of anaphylaxis, current guidance.
- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions, <https://www.nice.org.uk/guidance/mpg2>